ChemicalQDevice | Independent Research Paper | Draft 1.0

La Jolla Move-In: Pancreatic Oncology Clinical Trial Site Complete Documentation Package

15 Documents for California's First PDAC LLM Oncology Clinical Trial Site

Legislation, Regulations, Building Code, Premises Code, Conventional Trial Requirements, and Operations

Draft 1.0 | Repository v4.7.0 | 15 documents | 11 roles | $700,000 per year

01 SB 1188 Site Authorization	02 AB 3162 Patient Rights	03 SB 964 Data Protection	04 H. R. 10412 Federal Act	05 San Diego Municipal Code
06 Title 22 Chapter 15	07 FDA Compliance Guide	08 Building Code	09 Premises Code	10 Parking and Transport
11 Emergency Plan	12 Activation and SOPs	13 Trial Requirements	14 Staffing and Roles	15 Funding and Lobbying

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Independent research paper and practical adoption guide. It is not medical or regulatory advice and is not endorsed by the FDA, NIH, HHS, an IRB, ICH, or any sponsor. All figures derive from the author's repository sources and are illustrative unless tied to a cited reference.

Disclaimer: *This work is independent and is not endorsed or sponsored by any trial sponsor, CRO, site, IRB, regulator, or medical society; and was adapted using Claude Code Opus 5.*

The paper (v1.0) is at https://doi.org/10.5281/zenodo.xxxxxxxx and the repository (v4.7.0) is at https://github.com/kevinkawchak/physical-ai-oncology-trials. The paper files are deposited in /funding/move-in.

Keywords La Jolla; Pancreatic ductal adenocarcinoma; Phase 1 clinical trial site; Large language model governance; Robotic pancreaticoduodenectomy; Daraxonrasib; Site authorization; Building and premises code; Federal award stewardship; Legislative engagement

Table of Contents

Front Matter: The La Jolla Move-In 5
1 Why La Jolla, and Why Now . . . 5
2 How This Package Is Organized . . . 5
3 The Site at a Glance . . . 6
4 The Author Record and the Funding Position . . . 6
5 How to Read This Package . . . 6

1. SB 1188 - California PDAC LLM Oncology Clinical Trial Site Authorization and Site Establishment Act of 2026 7
1 Legislative Findings and Declarations . . . 7
2 Definitions . . . 7
3 Site Authorization . . . 7

4 Operating Conditions . . . 7
5 Inspection, Enforcement, and Suspension . . . 8
6 Reporting and Sunset . . . 8

2. AB 3162 - California LLM Oncology Patient Rights and Robotic Safety Act of 2026 9
1 Findings and Declarations . . . 9
2 Definitions . . . 9
3 Participant Rights . . . 9
4 Disclosure and Consent . . . 9
5 Robotic Safety Requirements . . . 9
6 Adverse Event Reporting and Remedies . . . 9

3. SB 964 - California Oncology Model Transparency and Clinical Data Protection Act of 2026 10
1 Findings and Declarations . . . 10
2 Definitions . . . 10
3 Model Transparency Obligations . . . 10
4 Clinical Data Protection . . . 10
5 Audit Trail and Retention . . . 10
6 Enforcement and Breach Notification . . . 10

4. H. R. 10412 - Independent Investigator Pancreatic Cancer Trial Site Act of 2026 11
1 Short Title and Findings . . . 11
2 Definitions . . . 11
3 Demonstration Site Designation . . . 11
4 Authorization of Appropriations . . . 11
5 FDA Review Timelines and Reporting . . . 11
6 Relationship to Existing Law . . . 11

5. San Diego Municipal Code Update - PDAC LLM Oncology Clinical Trial Site Requirements, La Jolla Community Plan Area 12
1 Purpose and Applicability . . . 12
2 Zoning and Land Use . . . 12
3 Conditional Use Permit Findings . . . 12
4 Coastal Overlay and Community Plan Consistency . . . 12
5 Operating Standards . . . 12
6 Permits, Fees, and Plan Check . . . 12

6. California Code of Regulations, Title 22, Division 5, Chapter 15 - PDAC LLM Oncology Clinical Trial Site Regulations 13
1 Scope and Authority . . . 13
2 Application for Authorization . . . 13
3 Staffing and Qualification Requirements . . . 13
4 Robot and Model Registration . . . 13
5 Survey, Deficiency Classes, and Penalties . . . 13
6 Records and Reporting . . . 13

7. FDA LLM and Robotic Workflow National Compliance Guide for a Phase 1 PDAC Clinical Trial Site 14
1 Purpose and Scope . . . 14
2 The Regulatory Position of the Site . . . 14
3 Correction Map to 21 CFR Parts 11, 50, 54, 312, and 812 . . . 14
4 ICH E6(R3) Mapping . . . 14
5 The Advisory-Only Boundary and Software Change Control . . . 14
6 Pre-Submission Sequence and Evidence Package . . . 14

8. Building Code Standards for a PDAC LLM Oncology Clinical Trial Facility 15
1 Scope, Occupancy, and Applicable Codes . . . 15
2 Site and Structural Provisions . . . 15
3 Mechanical, Electrical, and Plumbing . . . 15
4 Medical Gas, Shielding, and Specialty Systems . . . 15
5 Technology Infrastructure and the Machine Room . . . 15
6 Commissioning and Certificate of Occupancy . . . 15

9. PDAC LLM Oncology Clinical Trial Site Premises Code **16**
1 Scope and Zone Classification . . . 16
2 Access Control and Physical Security . . . 16
3 Robot Operating Envelopes and Interlocks . . . 16
4 Clean, Sterile, and Soiled Corridors . . . 16
5 Waste Streams and Housekeeping . . . 16
6 Signage, Wayfinding, and Occupant Conduct . . . 16

10. PDAC LLM Oncology Clinical Trial Site Parking and Patient Transportation Standards **17**
1 Scope and Applicability . . . 17
2 Stall Count, Mix, and Dimensions . . . 17
3 Accessible Parking and Path of Travel . . . 17
4 Drop-Off, Ambulance, and Loading Geometry . . . 17
5 Post-Procedure Transportation . . . 17
6 Transportation Demand Management and Monitoring . . . 17

11. PDAC LLM Oncology Clinical Trial Site Emergency Preparedness and Business Continuity Plan **18**
1 Scope and Command Structure . . . 18
2 Fire and Evacuation . . . 18
3 Seismic and Structural Events . . . 18
4 Power Failure and Utility Loss . . . 18
5 Cybersecurity Incident and Model Fault . . . 18
6 Business Continuity, Drills, and Training . . . 18

12. PDAC LLM Oncology Clinical Trial Site Activation Checklist and Standard Operating Procedures **19**
1 Activation Gates . . . 19
2 The Activation Checklist . . . 19
3 Standard Operating Procedure Index . . . 19
4 Delegation of Authority . . . 19
5 Training and Competency . . . 19
6 Quality Management and Deviation Handling . . . 19

13. Conventional Pancreatic Cancer Clinical Trial Requirements Manual **20**
1 Scope: What the Site Must Have Regardless of the Model . . . 20
2 Protocol, IND, and IRB . . . 20
3 Eligibility, Consent, and Enrollment . . . 20
4 Dose Escalation and Dose-Limiting Toxicity . . . 20
5 Pharmacy, Laboratory, Imaging, and Biospecimens . . . 20
6 Monitoring, Source Documentation, Safety Reporting, and Archive . . . 20
7 The Evidence Behind the Design . . . 21

14. Staffing, Role Delineation, and Move-In Plan for a Chief Executive and Ten Coworkers **22**
1 The Eleven Roles . . . 22
2 Qualifications and Delegated Tasks . . . 22
3 The Sponsor-Investigator Firewall . . . 22
4 Coverage, Call, and Cross-Training . . . 22
5 Cost of the Roster . . . 22
6 The Move-In Schedule . . . 23

15. Federal Funding Stewardship, Lobbying, and Legislative Engagement Plan **24**
1 The Award and Its Terms . . . 24
2 Stewardship Controls . . . 24
3 Lobbying: What Is Permitted and What Is Not . . . 24
4 The Legislative Engagement Plan . . . 24
5 Industry and Institutional Communications . . . 24
6 The Author Record That Supports the Ask . . . 25

Back Matter **26**
Abbreviations . . . 26
Data and Code Availability . . . 26
Author Contributions and Conflicts . . . 26

Citation 26
References 26

August 23, 2026

LA JOLLA MOVE-IN, FRONT MATTER

Front Matter: The La Jolla Move-In

1. Why La Jolla, and Why Now

This package stands up one clinical trial site in La Jolla, San Diego, for one disease and one protocol. It is the successor to the eleven-document San Francisco package, which described a general oncology site running on demand for twenty-four hours a day [1].

[DRAFTING INSTRUCTION] *Open `funding/move-in/inputs/READMES/README-Physical-AI-Oncology-Clinical-Trial-Site-Complete-Documentation-Package.md` and take the eleven-document roster and the simulation evidence base. State in two paragraphs what the La Jolla site keeps from it and what it drops, and why the change is a narrowing rather than a reduction. The key driver named in the master prompt, clause L, is the author's own body of work, the startup's funding documents, and the White House and presidential support for independent scientists; that driver is stated here and developed in document 15.*

[DRAFTING INSTRUCTION] *Open `funding/potential-partners/UC-San-Diego/README.md` for the twelve-step feasibility sequence, and `funding/potential-partners/UC-San-Diego/priority-steps.md` for the three positioning corrections. La Jolla is chosen because the intended partner of choice sits in the same community plan area. Say that plainly, and say equally plainly that no agreement of any kind exists.*

2. How This Package Is Organized

Fifteen documents in four parts. Each opens on its own page and can be read alone.

[DRAFTING INSTRUCTION] *Populate the document index below from the roster in `funding/move-in/README.md` §3. Fifteen rows, four parts, and one sentence per row saying what a reader gets from that document that they get nowhere else in the package.*

No	Part	Document	What only this document gives the reader
01	I	SB 1188, Site Authorization Act	to be written in full-move-in
02	I	AB 3162, Patient Rights and Robotic Safety Act	to be written in full-move-in
15	IV	Federal Funding Stewardship and Legislative Engagement Plan	to be written in full-move-in

Table 1. *The fifteen documents. Stage 1 carries three representative rows; stage 2 populates all fifteen from the roster named in the drafting instruction above.*

3. The Site at a Glance

[DRAFTING INSTRUCTION] *Fix the site parameters here, once, so that documents 05 through 12 can cite this table instead of restating dimensions. Derive the participant numbers from* `funding/pdac-funding-applications/final-apply/sections/sec-05-trial-evidence.tex` *and the protocol design from* `trial-protocol/` *and* `trial-ind/`*. Do not carry over the San Francisco throughput figures: a Phase 1 site that treats up to eighteen participants is not a 168-patient-per-day facility, and saying so is the point of the comparison.*

Parameter	Value	Where it is used
Gross floor area	to be fixed	Documents 08, 09
Robotic procedure suites	to be fixed	Documents 08, 09, 13
Parking stalls	to be fixed	Document 10

Table 2. *Site parameters, fixed once and cited thereafter. Stage 2 completes every row and no other document restates a dimension.*

4. The Author Record and the Funding Position

[DRAFTING INSTRUCTION] *Open* `funding/move-in/inputs/ChemicalQDevice_Accomplishments.docx` *and take the record from October 2021 forward. State the California limited liability company period, the count of artificial-intelligence-generated papers in 2026 against the total body of work, and the open-source demonstration practice. Prior technologies used before 2024 are described only where they explain how the current method was reached, which the master prompt permits and does not require. State the award assumption from clause H once, here, and develop it in document 15.*

5. How to Read This Package

[DRAFTING INSTRUCTION] *Write the audience table: a legislative staffer, a city planner, a state surveyor, an FDA reviewer, a funder, a site principal investigator, and a prospective participant. One row each, naming the two or three documents that person needs and the order to read them in.*

DOCUMENT 1 OF 15

SB 1188 - California PDAC LLM Oncology Clinical Trial Site Authorization and Site Establishment Act of 2026

1. Legislative Findings and Declarations

The Legislature finds and declares all of the following.

[DRAFTING INSTRUCTION] *Write ten lettered findings. Each one must carry a citation to an author work rather than an assertion. Draw the evidence from* `funding/pdac-funding-applications/final-apply/sections/sec-05-trial-evidence.tex` *for the simulation-to-observation chronology, from* `funding/move-in/inputs/READMES/README-LLM-Pancreatic-Oncology-Clinical-Trial-System-Large-Documents-Funding-and-AI-Peer-Review.md` *for the twenty-paper readiness record, and from* `funding/move-in/inputs/READMES/README-Physical-AI-Oncology-Clinical-Trial-Site-Complete-Documentation-Package.md` *for the predecessor site's operating record. Finding (a) states the disease burden and the resectable-setting gap. Finding (j) states that California has an opportunity to authorize the first such site and that the authorization is conditional and revocable.*

(a) Pancreatic ductal adenocarcinoma remains among the most lethal solid tumors, and the resectable setting is served by fewer trials than the metastatic setting. *Stage 2 completes this finding with its citation.*

(b) *Nine further findings, to be written in full-move-in.*

2. Definitions

[DRAFTING INSTRUCTION] *Define, in lettered subdivisions: LLM-directed clinical workflow; advisory-only output; robotic procedure suite; site safety officer; qualified site; department; sponsor-investigator; audit trail; and verification package. Take the advisory-only definition from* `regulatory/Adaption-21-CFR-Part-312/` *and the audit-trail definition from* `funding/move-in/draft-move-in/sections/sec-03-sb-964-data-protection.tex`*, so the two bills cannot drift.*

3. Site Authorization

[DRAFTING INSTRUCTION] *Write the authorization procedure as a numbered sequence with named timelines: application content, completeness review at thirty days, substantive review at ninety days, conditional authorization, and the first-participant hold. Cite* `regulatory/adaption-ich-e6r3/` *for the good clinical practice conditions the department may attach.*

Step	Action	Statutory clock	Consequence of the department's silence
1	Application filed	to be fixed	to be fixed
2	Completeness review	to be fixed	to be fixed

Table 3. *The authorization sequence. Stage 2 fixes every clock and states what happens when the department does not act, because a statute that is silent on inaction is a statute with no schedule.*

4. Operating Conditions

[DRAFTING INSTRUCTION] *Six conditions, each mapped to the document in this package that implements it: staffing to document 14, premises to document 09, model governance to document 03, safety reporting to document 13, emergency response to document 11, and record retention to document 03.*

5. Inspection, Enforcement, and Suspension

[DRAFTING INSTRUCTION] *Three deficiency classes with the department's remedy for each, matching the class definitions written in document 06 so a site cannot be class A under one instrument and class B under another.*

6. Reporting and Sunset

[DRAFTING INSTRUCTION] *An annual report to the Legislature with named contents, and a sunset five years from operative date unless extended. State the sunset in the same subdivision as the report, so the report is the evidence the extension turns on.*

DOCUMENT 2 OF 15

AB 3162 - California LLM Oncology Patient Rights and Robotic Safety Act of 2026

1. Findings and Declarations

[DRAFTING INSTRUCTION] *Eight lettered findings. Ground the participant-facing findings in* `funding/supplementary/` *and in the patient advocacy work at* `funding/move-in/inputs/READMES/README-LLM-Pancreatic-Oncology-Clinical-Trial-System-Large-Documents-Funding-and-AI-Peer-Review.md`*, paper 18. Ground the robotic safety findings in paper 10 of the same table, the ten-gate assurance suite. Do not repeat findings already made in document 01; cross-reference them instead.*

2. Definitions

[DRAFTING INSTRUCTION] *Define participant, advisory-only output, human-only pathway, robotic operating envelope, emergency stop, and reportable robotic event. Where a term is already defined in document 01 §2, state that it carries the same meaning rather than restating it, and say so in one sentence.*

3. Participant Rights

[DRAFTING INSTRUCTION] *Seven rights, each written so that a participant can tell whether it has been honored. The right to a human-only pathway is the one that costs the site money, so it is stated first and without qualification, and the operational consequence is carried into document 12's standard operating procedure index.*

4. Disclosure and Consent

[DRAFTING INSTRUCTION] *Take the consent architecture from* `regulatory/Adaption-21-CFR-Part-50/` *and state what must appear in the consent form about model involvement, what must be said aloud during the consent discussion, and what must be documented afterward. Compare against* `funding/move-in/inputs/READMES/README-ChemicalQDevice-Deposited-Abstracts-Corpus.md` *for the author's prior consent language and keep the wording consistent.*

Disclosure	Where it is made	Evidence that it was made
Model contributes advisory output	to be fixed	to be fixed
Human-only pathway available	to be fixed	to be fixed

Table 4. *Disclosure, its place, and its evidence. Stage 2 completes the table; every row must name a record that a monitor could ask for.*

5. Robotic Safety Requirements

[DRAFTING INSTRUCTION] *Write the safety envelope requirements against* `funding/move-in/draft-move-in/sections/sec-09-premises-code.tex` *so the bill and the premises code state the same numbers. Cite IEC 80601-2-77 for robotically assisted surgical equipment and ISO 13482 where a non-surgical assistive robot is in the envelope.*

6. Adverse Event Reporting and Remedies

[DRAFTING INSTRUCTION] *Reporting timelines aligned to 21 CFR 312.32 as adapted in* `regulatory/Adaption-21-CFR-Part-312/`*, and a remedy section limited to injunctive relief. State the limit and the reason for it in the same subdivision.*

DOCUMENT 3 OF 15

SB 964 - California Oncology Model Transparency and Clinical Data Protection Act of 2026

1. Findings and Declarations

[DRAFTING INSTRUCTION] *Six findings. Anchor them to the California instruments already on the books rather than inventing a policy rationale: AB 3030 on artificial intelligence in health care services, AB 489 on deceptive terms, SB 1120 on utilization review, and AB 2013 on training data transparency. State what this bill adds that those four do not reach, which is the clinical trial setting and the retention of prompts and completions as source records.*

2. Definitions

[DRAFTING INSTRUCTION] *Define model, model version, prompt, completion, hash-chained audit trail, provenance record, and secondary use. Take the audit-trail construction from* `national-platform/` *and keep the definition identical to the one used in document 07 §5.*

3. Model Transparency Obligations

Obligation	Frequency	Record that discharges it
Training data provenance disclosure	to be fixed	to be fixed
Model version register	to be fixed	to be fixed

Table 5. *Transparency obligations. Stage 2 completes the table against the model governance section of document 07.*

[DRAFTING INSTRUCTION] *Write five obligations. Each must be discharged by a record that exists, not by a policy statement. Cross-check the register requirement against* `funding/move-in/draft-move-in/sections/sec-07-fda-compliance-guide.tex` *so the state register and the federal change-control record are the same artifact rather than two.*

4. Clinical Data Protection

[DRAFTING INSTRUCTION] *De-identification standard, the prohibition on secondary use without specific consent, and the relationship to 45 CFR Part 164. State which of the two standards governs where they differ, and do not leave the reader to work it out.*

5. Audit Trail and Retention

[DRAFTING INSTRUCTION] *Retention periods aligned to 21 CFR 312.62 and to the archive requirement in document 13 §6. Where the periods differ, the longer governs; say so.*

6. Enforcement and Breach Notification

[DRAFTING INSTRUCTION] *Notification clock, the department's remedies, and the interaction with existing California breach law. Keep the deficiency classes identical to documents 01 and 06.*

DOCUMENT 4 OF 15

H. R. 10412 - Independent Investigator Pancreatic Cancer Trial Site Act of 2026

1. Short Title and Findings

[DRAFTING INSTRUCTION] *This is the successor to the author's own H. R. 9510 (Bill v5.0), which is at* `funding/move-in/inputs/READMES/README-LLM-Pancreatic-Oncology-Clinical-Trial-System-Large-Documents-Funding-and-AI-Peer-Review.md`*, paper 12, and was iterated over four prior full works. Open by saying what H. R. 9510 did and what this bill adds, which is the site designation and the review-timeline reporting requirement. Do not restate the financial provisions of the predecessor; cite them.*

[DRAFTING INSTRUCTION] *Ground the findings in the two policy documents already in the repository:* `funding/science-golden-age/` *for the individual-scientist realignment of the federal research portfolio, and the annexed fiscal year 2028 budget priorities for long-duration grants and non-federal cost share. Clause L of the master prompt makes White House and presidential support for independent scientists a key driver; the findings are where that driver becomes statutory language.*

2. Definitions

[DRAFTING INSTRUCTION] *Define independent investigator, designated demonstration site, LLM-directed clinical workflow, and covered review action. Keep the workflow definition identical to document 01 §2 and document 07 §2.*

3. Demonstration Site Designation

Sec	Provision	What it obligates, and of whom
101	Designation authority	to be fixed
102	Eligibility criteria	to be fixed

Table 6. *The bill's operative sections. Stage 2 completes the table and every row names the officer or agency that carries the obligation.*

4. Authorization of Appropriations

[DRAFTING INSTRUCTION] *Size the authorization against the frame already fixed in* `funding/pdac-funding-applications/final-apply/sections/sec-08-budget-and-leverage.tex`*: $700,000 per year and $3,500,000 over five years for a single site. Do not re-derive the number. State how many sites the authorization would carry and show the arithmetic.*

5. FDA Review Timelines and Reporting

[DRAFTING INSTRUCTION] *The reporting requirement is the operative provision for clause C of the master prompt: making the LLM and robotic workflow reviewable on a schedule. Write it against* `regulatory/Adaption-21-CFR-Part-312/` *and* `trial-ind/`*, and state what the report must contain, to whom, and how often.*

6. Relationship to Existing Law

[DRAFTING INSTRUCTION] *A savings clause and an express statement that nothing in the bill displaces 21 CFR Parts 50, 312 or 812, or the Common Rule. Name each.*

DOCUMENT 5 OF 15

San Diego Municipal Code Update - PDAC LLM Oncology Clinical Trial Site Requirements, La Jolla Community Plan Area

1. Purpose and Applicability

[DRAFTING INSTRUCTION] *State the purpose in two sentences and the applicability in one table: which zones, which community plan area, and which permit type. Cite the San Diego Municipal Code and the La Jolla Community Plan by name. Do not invent a street address; the site is described by zone and by community plan area, and the address is stated to be withheld until a lease is executed.*

2. Zoning and Land Use

Zone	Use status	Permit	Condition attached by this update
Commercial office	to be fixed	to be fixed	to be fixed
Mixed use	to be fixed	to be fixed	to be fixed

Table 7. *Zone by zone, the use status and the permit. Stage 2 completes every row; a use that is not permitted anywhere in the community plan area is stated as such rather than omitted.*

3. Conditional Use Permit Findings

[DRAFTING INSTRUCTION] *Write the findings a hearing officer must make, in the form San Diego uses: consistency with the community plan, no adverse effect on the health, safety and general welfare of the surrounding area, and compliance with the regulations of the land development code. Add two findings specific to this use: robotic operating envelopes contained wholly within the building, and a model governance record available to the city on request.*

4. Coastal Overlay and Community Plan Consistency

[DRAFTING INSTRUCTION] *Address the coastal overlay and the California Coastal Act, and state what a coastal development permit would and would not require for a tenant improvement inside an existing shell. Cite the Coastal Act and the California Environmental Quality Act, and state which exemption is relied on and why.*

5. Operating Standards

[DRAFTING INSTRUCTION] *Hours of operation, noise limits at the property line, exterior lighting, delivery windows, and medical waste pickup. Keep the hours identical to document 12 §1 and the waste streams identical to document 09 §5. The site is extended-day, not twenty-four-hour; state the difference from the San Francisco predecessor explicitly and say why a Phase 1 site does not need around-the-clock operation.*

6. Permits, Fees, and Plan Check

[DRAFTING INSTRUCTION] *Sequence the permits: building, mechanical, electrical, plumbing, fire, hazardous materials, and the county health plan check. State which run in parallel and which are strictly sequential, because the move-in schedule in document 14 §6 is built on that ordering.*

DOCUMENT 6 OF 15

California Code of Regulations, Title 22, Division 5, Chapter 15 - PDAC LLM Oncology Clinical Trial Site Regulations

1. Scope and Authority

[DRAFTING INSTRUCTION] *State the enabling authority as document 01 and the Health and Safety Code division that licensing provisions sit in. Say what kind of facility this chapter regulates and what it does not: it is not a general acute care hospital chapter and does not displace one.*

2. Application for Authorization

[DRAFTING INSTRUCTION] *Enumerate the application contents as lettered subdivisions, and make each one an artifact that exists elsewhere in this package: the staffing plan is document 14, the premises plan is document 09, the emergency plan is document 11, and the model governance record is document 03. A regulation whose application asks for documents that do not exist is a regulation that cannot be complied with.*

3. Staffing and Qualification Requirements

Position	Minimum	Qualification the department will verify
Site principal investigator	to be fixed	to be fixed
Site safety officer	to be fixed	to be fixed

Table 8. *Staffing minimums. Stage 2 completes the table from document 14 §1 and §2, and the two must agree row for row.*

4. Robot and Model Registration

[DRAFTING INSTRUCTION] *A register with a row per robot instance and a row per model version. Take the instance count from the site parameter table in the front matter and the model version fields from document 03 §3. State the event that triggers an amendment to the register and the clock for filing it.*

5. Survey, Deficiency Classes, and Penalties

[DRAFTING INSTRUCTION] *Three classes, a defined remedy per class, and a civil penalty range per class. Keep the class definitions identical to document 01 §5. State the appeal path.*

6. Records and Reporting

[DRAFTING INSTRUCTION] *Annual survey report, event reporting, and the retention period. Align the retention period with document 03 §5 and document 13 §6, and where they differ, state that the longest governs.*

DOCUMENT 7 OF 15

FDA LLM and Robotic Workflow National Compliance Guide for a Phase 1 PDAC Clinical Trial Site

1. Purpose and Scope

[DRAFTING INSTRUCTION] *Say what this document is and what it is not. It is a map from the site to the federal requirements that already exist. It is not a submission, not a guidance document, and not an FDA position. State that in the first paragraph, because the master prompt's clause C asks for the workflow to be acceptable to the agency and the honest form of that ask is a map, not a claim.*

2. The Regulatory Position of the Site

[DRAFTING INSTRUCTION] *Take the combined investigational new drug and investigational device exemption position from* `trial-protocol/` *and* `trial-ind/`*, and the filing itself from paper 16 of* `funding/move-in/inputs/READMES/README-LLM-Pancreatic-Oncology-Clinical-Trial-System-Large-Documents-Funding-and-AI-Peer-Review.md`*, the 172-page Investigational New Drug Application. State which parts of the workflow sit under the drug filing, which under the device pathway, and which under neither because they are practice of medicine.*

3. Correction Map to 21 CFR Parts 11, 50, 54, 312, and 812

Citation	Requirement	Where this site discharges it
21 CFR 11.10	to be fixed	to be fixed
21 CFR 312.62	to be fixed	to be fixed

Table 9. *The correction map. Stage 2 completes it from the three adapted frameworks at* `regulatory/`*, one row per requirement, and every row names a document in this package.*

[DRAFTING INSTRUCTION] *Build the map from* `regulatory/adaption-ich-e6r3/`*,* `regulatory/Adaption-21-CFR-Part-50/` *and* `regulatory/Adaption-21-CFR-Part-312/`*. Add 21 CFR Part 11 for the electronic record and signature position and 21 CFR Part 54 for the financial disclosure position, which is the one that decides the sponsor-investigator firewall in document 14 §3.*

4. ICH E6(R3) Mapping

[DRAFTING INSTRUCTION] *Map the site to the good clinical practice principles, using the author's own adaptation rather than the source guideline where the two differ, and say when they differ.*

5. The Advisory-Only Boundary and Software Change Control

[DRAFTING INSTRUCTION] *This is the load-bearing section of the document. State the boundary as a rule that can be tested: what the model may produce, what a human must do before any of it reaches a participant, and what record proves the human acted. Take the verification-before-generation position from paper 11 and the supervision rationale from paper 13 of the deck table. State the change control plan against the FDA predetermined change control plan guidance.*

6. Pre-Submission Sequence and Evidence Package

[DRAFTING INSTRUCTION] *Order the pre-submission steps and name the evidence each one carries, drawing the credibility evidence from* `funding/pdac-funding-applications/final-apply/sections/sec-05-trial-evidence.tex`*: the ASME V&V 40 and ICH M15 aligned credibility score of 81.9 across 55 verification notebook tests. Carry the limitation each source stated in the same row as its result.*

DOCUMENT 8 OF 15

Building Code Standards for a PDAC LLM Oncology Clinical Trial Facility

1. Scope, Occupancy, and Applicable Codes

[DRAFTING INSTRUCTION] *Fix the occupancy classification and name every code that governs: the California Building Standards Code, Title 24, NFPA 99 for health care facilities, and NFPA 101 for life safety. State which parts of the facility are business occupancy and which are ambulatory care, because the two carry different sprinkler and egress requirements and the difference decides the tenant improvement cost in document 14 §6.*

2. Site and Structural Provisions

Element	Requirement	Why this trial needs it
Procedure suite floor loading	to be fixed	to be fixed
Vibration limit at the robot base	to be fixed	to be fixed

Table 10. *Structural provisions. Stage 2 fixes each value and states the consequence of not meeting it, which for a robot base is a registration error rather than a structural failure.*

[DRAFTING INSTRUCTION] *Derive the robotic arm base and vibration provisions from the device envelope described in papers 9 and 10 of the deck table at* `funding/move-in/inputs/READMES/README-LLM-Pancreatic-Oncology-Clinical-Trial-System-Large-Documents-Funding-and-AI-Peer-Review.md`*. Cite IEC 80601-2-77 for robotically assisted surgical equipment.*

3. Mechanical, Electrical, and Plumbing

[DRAFTING INSTRUCTION] *Air changes per hour and pressure relationships for the procedure suites, the pharmacy compounding area, and the specimen laboratory; essential electrical system branches; and the plumbing provisions for the compounding area. Take the pharmacy provisions from USP General Chapters 797 and 800, and keep them identical to document 13 §5.*

4. Medical Gas, Shielding, and Specialty Systems

[DRAFTING INSTRUCTION] *Medical gas outlets per suite, imaging shielding where a fluoroscopic or computed tomography unit is present, and the specialty systems the protocol requires. If the protocol does not require an imaging modality on site, say so and state where the imaging is performed instead, citing* `trial-protocol/`*.*

5. Technology Infrastructure and the Machine Room

[DRAFTING INSTRUCTION] *The on-premises model node is the reason this document differs from an ordinary ambulatory surgical center building code. Fix the machine room provisions: power, cooling, physical access, network segmentation, and the uninterruptible supply runtime. Keep the segmentation identical to document 09 §2 and document 11 §5. Take the on-premises requirement from* `trial-protocol/` *rather than asserting it.*

6. Commissioning and Certificate of Occupancy

[DRAFTING INSTRUCTION] *The commissioning sequence as a gated table, ending at the certificate of occupancy, and the handoff to the activation checklist in document 12 §2. The two must share a boundary: the last commissioning gate is the first activation gate.*

DOCUMENT 9 OF 15

PDAC LLM Oncology Clinical Trial Site Premises Code

1. Scope and Zone Classification

[DRAFTING INSTRUCTION] *Define the premises zones and state which people and which robots may enter each. Four zones is the working assumption: public, clinical, restricted and machine. Fix the assumption here and use it unchanged in documents 10, 11 and 12.*

Zone	Rooms	Who may enter	Credential and record
Public	to be fixed	to be fixed	to be fixed
Machine	to be fixed	to be fixed	to be fixed

Table 11. *The four premises zones. Stage 2 completes the table, and every row names the credential and the access record, because a zone with no record is a zone with no control.*

2. Access Control and Physical Security

[DRAFTING INSTRUCTION] *Credentialing, badge classes, visitor escort, after-hours access, and the retention of access logs. Align the log retention with document 03 §5.*

3. Robot Operating Envelopes and Interlocks

[DRAFTING INSTRUCTION] *This is the section document 02 §5 points at, so the numbers must be identical. Fix the envelope boundary, the interlock chain, the emergency stop reach distance, and the recovery procedure after an emergency stop. Cite IEC 80601-2-77 and ISO 13482.*

4. Clean, Sterile, and Soiled Corridors

[DRAFTING INSTRUCTION] *Traffic patterns, the separation of clean from soiled, and the handling of an instrument tray between the two. Keep the pattern consistent with the air pressure relationships fixed in document 08 §3.*

5. Waste Streams and Housekeeping

[DRAFTING INSTRUCTION] *Five streams: general, recyclable, regulated medical, cytotoxic and pharmaceutical, and electronic. State the container, the storage location, the pickup interval and the manifest for each. Keep the pickup windows identical to document 05 §5.*

6. Signage, Wayfinding, and Occupant Conduct

[DRAFTING INSTRUCTION] *Signage required at each zone boundary, the wayfinding path from the parking structure to the clinical zone, which document 10 §3 also describes, and the conduct rules that apply to occupants inside a robot envelope.*

DOCUMENT 10 OF 15

PDAC LLM Oncology Clinical Trial Site Parking and Patient Transportation Standards

1. Scope and Applicability

[DRAFTING INSTRUCTION] *State what this document governs and how it relates to the San Diego Municipal Code parking requirements in document 05. Where the city minimum and the trial requirement differ, the higher governs; say so once here.*

2. Stall Count, Mix, and Dimensions

Stall class	Count	Dimension	Basis for the count
Participant	to be fixed	to be fixed	to be fixed
Accessible	to be fixed	to be fixed	to be fixed

Table 12. *The stall schedule. Stage 2 completes it and the count column sums to the total fixed in the site parameter table in the front matter.*

[DRAFTING INSTRUCTION] *Derive the participant stall count from the visit schedule in* `trial-protocol/` *rather than from a floor area ratio: a Phase 1 site with up to eighteen treated participants has a peak concurrency that can be counted, and counting it is more defensible than applying a general medical office ratio.*

3. Accessible Parking and Path of Travel

[DRAFTING INSTRUCTION] *Accessible stall count and the accessible route, against the 2010 ADA Standards for Accessible Design and Title 24. State the path of travel from the accessible stall to the clinical zone entrance and keep it identical to the wayfinding path in document 09 §6.*

4. Drop-Off, Ambulance, and Loading Geometry

[DRAFTING INSTRUCTION] *Turning radius, canopy clearance, and the separation of the ambulance approach from the participant drop-off. State the clearance that a transfer ambulance needs and cite document 11 §2 for the evacuation route it shares.*

5. Post-Procedure Transportation

[DRAFTING INSTRUCTION] *A participant discharged after a pancreaticoduodenectomy does not drive. State the requirement, the responsible role from document 14, the record that proves the arrangement was made, and what happens when it fails. Take the participant-facing framing from paper 18 of the deck table.*

6. Transportation Demand Management and Monitoring

[DRAFTING INSTRUCTION] *The measures the city would expect and the monitoring report that demonstrates them. Keep the reporting interval identical to document 05 §6.*

DOCUMENT 11 OF 15

PDAC LLM Oncology Clinical Trial Site Emergency Preparedness and Business Continuity Plan

1. Scope and Command Structure

[DRAFTING INSTRUCTION] *Name the incident commander, the alternate, and the succession, using the roles fixed in document 14 §1 rather than inventing titles. State the one decision that may not be delegated, which is the decision to stop a procedure in progress.*

2. Fire and Evacuation

[DRAFTING INSTRUCTION] *Alarm, response, evacuation routes, assembly, and the special case of a participant under anesthesia. Cite NFPA 101 and keep the routes identical to the egress plan in document 08 §1.*

3. Seismic and Structural Events

[DRAFTING INSTRUCTION] *Immediate actions, the post-event inspection sequence, and the condition on which robotic operations may resume. State that resumption requires a re-verification of the robot registration, and cite the registration requirement in document 06 §4.*

4. Power Failure and Utility Loss

System	Backup	Runtime	Action at exhaustion
Procedure suite	to be fixed	to be fixed	to be fixed
Machine room	to be fixed	to be fixed	to be fixed

Table 13. *Backup power by system. Stage 2 fixes the runtimes against the essential electrical provisions in document 08 §3, and states the action at exhaustion for every row.*

5. Cybersecurity Incident and Model Fault

[DRAFTING INSTRUCTION] *Two distinct events with two distinct responses: a network compromise and a model producing output outside its verified envelope. Take the fault taxonomy from paper 11 of the deck table and the response structure from the NIST Cybersecurity Framework 2.0. State that a model fault does not stop the trial; it stops the advisory output and the human pathway continues, which is the whole point of the advisory-only boundary in document 07 §5.*

6. Business Continuity, Drills, and Training

[DRAFTING INSTRUCTION] *Recovery point and recovery time objectives, the mutual aid arrangement for participant transfer, the quarterly drill rotation, and the training requirement. Keep the training hours identical to document 12 §5.*

DOCUMENT 12 OF 15

PDAC LLM Oncology Clinical Trial Site Activation Checklist and Standard Operating Procedures

1. Activation Gates

[DRAFTING INSTRUCTION] *Five gates, each with a stop condition that is a signed instrument or a number rather than a judgment. Adapt the gate discipline from the twelve- milestone schedule in* `funding/capitalization-plan/final-capital/sections/`*, which states a stop condition before the work begins. Gate 5 is first-participant-in and it may not open before the institutional review board approval and the investigational new drug safe-to-proceed are both in hand.*

2. The Activation Checklist

☐	Item	Owner	Evidence that closes it
☐	Certificate of occupancy issued	to be fixed	to be fixed
☐	Robot registration filed	to be fixed	to be fixed

Table 14. *The activation checklist. Stage 2 writes every item, and every owner is one of the eleven roles in document 14 §1, never a department name.*

3. Standard Operating Procedure Index

[DRAFTING INSTRUCTION] *Index the standard operating procedures the site needs, one row each, with an owner and a review interval. The index must cover both the conventional procedures document 13 requires and the model-specific procedures document 07 §5 requires, and the two groups must be visually distinguishable in the table so a monitor can see which is which.*

4. Delegation of Authority

[DRAFTING INSTRUCTION] *The delegation of authority log, its required fields, and the rule that a task may not be performed before the log entry exists. Cross-reference document 14 §2, which lists the delegated tasks per role.*

5. Training and Competency

[DRAFTING INSTRUCTION] *Training hours by role, the competency assessment, and the re-training trigger. State the hours once here; documents 06 and 11 cite this section rather than restating them.*

6. Quality Management and Deviation Handling

[DRAFTING INSTRUCTION] *Deviation classes, the reporting clock, the corrective and preventive action loop, and the quarterly quality review. Align the deviation classes with the deficiency classes in documents 01, 03 and 06 so a single event is not graded on three scales.*

DOCUMENT 13 OF 15

Conventional Pancreatic Cancer Clinical Trial Requirements Manual

1. Scope: What the Site Must Have Regardless of the Model

[DRAFTING INSTRUCTION] *Open with the test this document is written to pass: a reader who deletes every large language model and every robot from the site still holds a complete conventional Phase 1 pancreatic cancer requirements manual. Say that in the first paragraph and hold to it for the whole document. Nothing in this document may depend on the model.*

2. Protocol, IND, and IRB

[DRAFTING INSTRUCTION] *Take the protocol from* `trial-protocol/` *and the filing from* `trial-ind/`*. State the design in one paragraph: open-label, single-arm, 3+3 dose escalation, up to eighteen treated participants, perioperative daraxonrasib with a staged robotic pancreaticoduodenectomy. State the positioning correction from* `funding/potential-partners/UC-San-Diego/priority-steps.md` *in the same paragraph: daraxonrasib is investigational and already in Phase 3 evaluation, and is not described as first in human.*

3. Eligibility, Consent, and Enrollment

[DRAFTING INSTRUCTION] *Inclusion and exclusion criteria as two lettered lists, the consent process, and the enrollment log. Keep the consent architecture identical to document 02 §4 and cite* `regulatory/Adaption-21-CFR-Part-50/`*.*

4. Dose Escalation and Dose-Limiting Toxicity

Level	Dose	Participants	Escalation rule
−1	to be fixed	to be fixed	to be fixed
1	to be fixed	to be fixed	to be fixed

Table 15. *The escalation table. Stage 2 completes it from the protocol and states the dose-limiting toxicity window explicitly, because a 3+3 design with an unstated window is not a design.*

[DRAFTING INSTRUCTION] *Define dose-limiting toxicity against CTCAE version 5.0 and the surgical complication grading against Clavien-Dindo, and grade postoperative pancreatic fistula against the 2016 International Study Group of Pancreatic Surgery definition. Name all three.*

5. Pharmacy, Laboratory, Imaging, and Biospecimens

[DRAFTING INSTRUCTION] *Four subsections. Pharmacy: accountability under 21 CFR 312.57 and 312.62 and compounding under USP 797 and 800. Laboratory: certification under the Clinical Laboratory Improvement Amendments. Imaging: response assessment under RECIST 1.1 and the acquisition schedule. Biospecimens: collection, processing, storage and chain of custody. Keep every facility requirement identical to document 08 §3.*

6. Monitoring, Source Documentation, Safety Reporting, and Archive

[DRAFTING INSTRUCTION] *Monitoring plan, source data verification, the safety reporting clocks under 21 CFR 312.32, and the archive period. Keep the archive period aligned with document 03 §5 and document 06 §6.*

7. The Evidence Behind the Design

[DRAFTING INSTRUCTION] *This is the section a funder reads. Build the four-source evidence table from* `funding/pdac-funding-applications/final-apply/sections/sec-05-trial-evidence.tex`*: the 100,000-patient triplicate, the ten-arm quantitative systems pharmacology run at 12.8 months median overall survival against 5.4, the 1000-patient digital twin at a progression-free survival hazard ratio of 0.31, and RASolute 302 at 13.2 months against 6.6. Carry each source's own stated limitation in the same row as its result. State the 2.4-fold against 2.0-fold proximity as a chronology observation and not a validation claim, and name the three material differences. Add the cost benchmark table from* `funding/capitalization-plan/final-capital/sections/sec-06-clinical-evidence.tex`*, with the $36,330 figure described as projected and never as estimated.*

DOCUMENT 14 OF 15

Staffing, Role Delineation, and Move-In Plan for a Chief Executive and Ten Coworkers

1. The Eleven Roles

No	Role	Award FTE	The one thing the site cannot do without this role
1	Chief Executive Officer and sponsor representative	0.20	to be fixed
5	Lead clinical research coordinator	1.00	to be fixed

Table 16. *The eleven roles. Stage 2 completes all eleven rows and the full-time equivalent column sums to 3.95, which is checked arithmetically rather than asserted.*

[DRAFTING INSTRUCTION] *Complete the roster from the table in* `funding/move-in/sub-prompts/full-move-in/prompt-4-fifteen-documents.md`*, which fixes the eleven roles and their full-time equivalent fractions. Do not change a fraction without changing the cost table in §5 in the same edit.*

2. Qualifications and Delegated Tasks

[DRAFTING INSTRUCTION] *One row per role: the qualification the department will verify under document 06 §3, and the tasks delegated under document 12 §4. The two tables must agree, and the qualification column must match document 06 word for word.*

3. The Sponsor-Investigator Firewall

[DRAFTING INSTRUCTION] *State the 21 CFR 54 position plainly. The chief executive is chief executive of the sponsor and holds 100 percent of it, so every trigger in 21 CFR 54.2 would fire if he were also a clinical investigator. The investigator role therefore sits wholly with the site. Take the wording from* `funding/capitalization-plan/final-capital/sections/` *and keep it consistent, including the point that a Phase 1 3+3 escalation carries no randomization, so the firewall binds on dose assignment, endpoint adjudication and the analysis plan instead.*

4. Coverage, Call, and Cross-Training

[DRAFTING INSTRUCTION] *A roster of 3.95 award-funded full-time equivalents cannot cover a twenty-four-hour service, and the site does not attempt one. State the coverage window, the call arrangement for a post-procedure participant, and which two roles are cross-trained for which tasks. Keep the window identical to document 05 §5.*

5. Cost of the Roster

Role	FTE	Charged per year	Note
Lead clinical research coordinator	1.00	to be fixed	to be fixed

Table 17. *The cost of the roster. Stage 2 completes it so the charged column sums to $521,000 and the remaining $179,000 of the annual $700,000 is accounted for in document 15 §1.*

[DRAFTING INSTRUCTION] *Reuse the budget frame from* `funding/pdac-funding-applications/final-apply/sections/sec-08-budget-and-leverage.tex`*: $700,000 per year and $3,500,000 over five years. Do not re-derive it. Show the arithmetic that splits it into personnel and non-personnel.*

6. The Move-In Schedule

[DRAFTING INSTRUCTION] *A week-by-week schedule from lease execution to first participant in, with the eleven roles entering on the week each one is first needed. Order the permit sequence exactly as document 05 §6 states it, because the schedule depends on which permits run in parallel.*

DOCUMENT 15 OF 15

Federal Funding Stewardship, Lobbying, and Legislative Engagement Plan

1. The Award and Its Terms

[DRAFTING INSTRUCTION] *State the award assumption from clause H of the master prompt once, plainly: $700,000 per year for five years, $3,500,000 in direct cost, from at least one federal agency. Reuse the four-layer split from* `funding/pdac-funding-applications/final-apply/sections/sec-08-budget-and-leverage.tex` *and the indirect-cost position from* `funding/capitalization-plan/final-capital/sections/`. *Complete the six-line budget so it sums to $700,000 exactly, and reconcile the personnel line with document 14 §5.*

Line	Per year	Five years	Basis
Personnel, 3.95 award-funded FTE	$521,000	$2,605,000	Document 14 §5
Total direct cost	$700,000	$3,500,000	to be fixed

Table 18. *The award, line by line. Stage 2 completes the six lines and the column sums are checked, not asserted.*

2. Stewardship Controls

[DRAFTING INSTRUCTION] *Six controls, each with the record that evidences it: separation of duties, prior approval thresholds, effort certification, subaward monitoring, equipment tagging, and the annual financial report. Cite 2 CFR 200.414 for the indirect-cost position.*

3. Lobbying: What Is Permitted and What Is Not

[DRAFTING INSTRUCTION] *This section has to be exact. Federal award funds may not be used for lobbying under 2 CFR 200.450, and 31 U.S.C. 1352 restricts the use of appropriated funds to influence federal awarding actions. State what the company may therefore do with award funds, what it may do with non-federal funds, and which activity in the legislative engagement plan below is funded from which source. A plan that does not separate the two is not usable.*

Activity	Funding source	Authority
Technical assistance at a legislative request	to be fixed	to be fixed
Advocacy for enactment of documents 01 to 04	to be fixed	to be fixed

Table 19. *Activity, source and authority. Stage 2 completes the table, and no row may show a federal source beside an advocacy activity.*

4. The Legislative Engagement Plan

[DRAFTING INSTRUCTION] *One plan per instrument: documents 01, 02 and 03 to the California Legislature, and document 04 to Congress. For each, name the committee of jurisdiction by function rather than by member name, the artifact to be shared, and the sequence. Ground the federal engagement in* `funding/science-golden-age/` *and in the predecessor bill H. R. 9510.*

5. Industry and Institutional Communications

[DRAFTING INSTRUCTION] *Clause G of the master prompt records that industry recipients of communications have been responsive. State what has and has not happened in terms a reader can check, and repeat the three positioning corrections from* `funding/potential-partners/UC-San-Diego/priority-steps.md`*: no drug* *supply agreement or letter of authorization exists, no robotic configuration is specified before the site agreement, and no institutional agreement of any kind is in place.*

6. The Author Record That Supports the Ask

[DRAFTING INSTRUCTION] *Clauses D, E, F and L. Build the record from* `funding/move-in/inputs/ChemicalQDevice_Accomplishments.docx` *and the twenty-paper table in* `funding/move-in/inputs/READMES/README-LLM-Pancreatic-Oncology-Clinical-Trial-System-Large-Documents-Funding-and-AI-Peer-Review.md`*. Give the twenty rows as a table with deposit date, identifier and the requirement the La Jolla site inherits from that paper. State the favorable federal funder responses, the three presidential chief executive recognition letters, and the White House support for independent scientists as facts of record, each in one sentence, without embellishment.*

LA JOLLA MOVE-IN, BACK MATTER

Back Matter

Abbreviations

[DRAFTING INSTRUCTION] *Build the abbreviations table in two column pairs at the body measure. The rule for inclusion is mechanical: every abbreviation used more than once in the body appears here, and every row here is used in the body. Stage 3 audits the table against the body in both directions.*

Short	Expansion	Short	Expansion
AE	Adverse event	IRB	Institutional review board
CFR	Code of Federal Regulations	LLM	Large language model
PDAC	Pancreatic ductal adenocarcinoma	SOP	Standard operating procedure

Table 20. *Abbreviations, in two column pairs at the body measure. Stage 1 carries three representative pairs; stage 2 completes the table.*

Data and Code Availability

Every work cited to a digital object identifier is openly deposited. The repository is at `https://github.com/kevinkawchak/physical-ai-oncology-trials`, version v4.7.0 [2].

[DRAFTING INSTRUCTION] *Complete the availability paragraph: name the three stage directories, the two prompt files, the fifteen sub-prompts, and the input artifacts. State that no raster image is used anywhere in the package, which is Rule 3, and that the statement can be checked by searching the subtree for `.png` and `.jpg`.*

Author Contributions and Conflicts

[DRAFTING INSTRUCTION] *Kevin Kawchak is the sole author. State the 21 CFR Part 54 position in the same paragraph as the conflict statement, because the two are the same fact seen from two sides: he is chief executive of the sponsor, holds 100 percent of it, and is therefore not a clinical investigator on this trial. State that no sponsor, contract research organization, institutional review board, regulator or medical society reviewed this package or authorized any statement in it, and that the work was adapted using Claude Code Opus 5.*

Citation

Kawchak K. La Jolla Move-In: Pancreatic Oncology Clinical Trial Site Complete Documentation Package. Zenodo. 2026; 10.5281/zenodo.xxxxxxxx.

References

[1] Kevin Kawchak. Physical AI Oncology Clinical Trial Site Complete Documentation Package. Zenodo, v2.9.0, eleven documents, March 2026. doi:10.5281/zenodo.19176370.

[2] Kevin Kawchak. physical-ai-oncology-trials. GitHub repository, v4.7.0, August 2026. URL: https://github.com/kevinkawchak/physical-ai-oncology-trials.